NCCP 00545 · VERSION 3

Zoledronic acid · adjuvant breast treatment

Zoledronic acid by IV infusion once every six months for up to five years, unless progression or unacceptable toxicity changes the plan. The usual source dose is 4 mg, with renal review before prescribing.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Kidney injury and low calcium

☐ Flu-like reaction and muscle or joint pain

☐ Uncommon osteonecrosis of the jaw

☐ Rare atypical thigh fracture or eye inflammation

Discuss the adjuvant off-label indication, renal risk, hypocalcaemia, jaw osteonecrosis, rare atypical fracture and alternatives. Record menopause status and the agreed duration.

Individual benefit, alternatives and concerns

Agreed checks and next review

Before starting: renal profile, magnesium, calcium and phosphate. Check creatinine before every infusion; compare it with baseline as well as reviewing creatinine clearance. Correct low calcium and review vitamin D. Complete a dental assessment and preventive care before treatment where appropriate. Report dental symptoms; discuss invasive dental work with oncology and dental teams. Hydration is individualised, especially with heart failure.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.